

Chapter 2 Review:

- Verbal vs non-verbal communication
- Objective information vs subjective information
- Senses that are used in observation
- Barriers to communicating with residents
- How to avoid barriers to communicating
- Cliche/slang use
- Medical abbreviations (and when they can be used)
- How to assist a resident who is visually impaired
- How to assist a resident who is hearing impaired
- How to interact with residents with a mental disorder
- How to deal with a combative resident
- Proper body mechanics (lifting and how resident should be positioned for eating)
- Factors that increase the risk for resident falls
- OSHA
- MDS
- Fire safety (RACE and PASS)
- Responding to non-medical emergencies
- Responding to medical emergencies-assess the situation, assess the victim
- Universal sign for choking
- CPR-abdominal thrusts
- Shock
- Myocardial infarction (MI also known as heart attack)
- Dyspnea
- Controlling bleeding
- Treating burns
- Syncope
- Insulin reaction (things that can cause it)
- Diabetic ketoacidosis (what breath smells like)
- Responding to seizures
- CVA vs TIA (acronym FAST)
- Emesis
- Localized vs systemic infection
- Chain of infection
- Standard Precautions
- When to wear gloves

- Steps of handwashing technique
- Steps of donning and doffing PPE
- Parts of words (prefix, suffix, root)
- Modes of transmission/Types of isolation/precautions (bloodborne, airborne, droplet, contact)
- Hepatitis, TB, MRSA, VRE, C diff